



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : رباح نادى ضاحى حسن
المحافظة : المنيا
الحى / المدينة : دير مواس
رقم البطاقة : 29207272400329 رقم التليفون : 01125887674
العنوان : العماره الشرقيه
العمر (تاريخ الميلاد) : 27-07-1992 المهنة : حاصل علي دبلوم
تاريخ تسجيل الإستماره : 22-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج كيماوي تاكسان (دوسيتاكل) قسم العلاج :
[14152]
الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: رباح نادى ضاحي حسن National ID: 29207272400329	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Docetaxel chemotherapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

رباح
نادى ضاحى حسن

العماريه الشرقيه
مركز ديرمواس - المنيا

١٩٩٢/٠٧

LQ3813893

٢٩٢٠٧٢٧٢٤٠٠٣٢٩



٢٠٢٦/٠٢ ٢٩٢٠٧٢٧٢٤٠٠٣٢٩

دبلوم تجاره

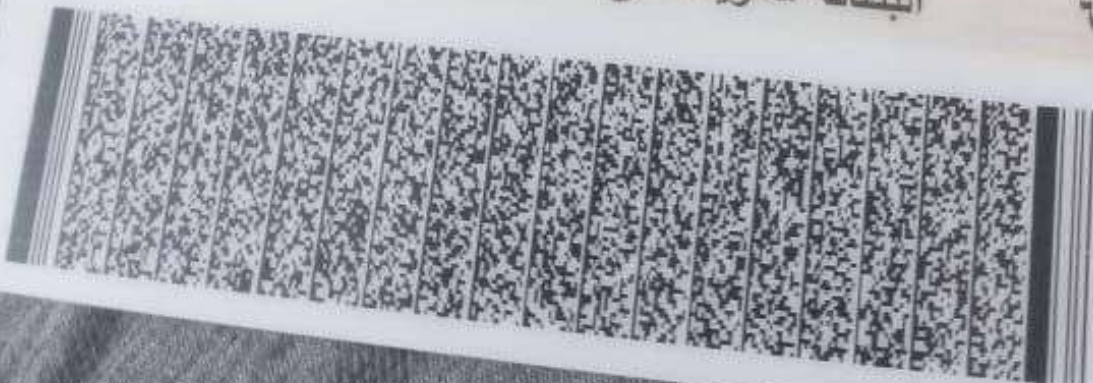


أنثى مسلمة متزوجة

هانى هاشم حسين مزيون

البطاقة سارية حتى ٢٠٣٣/٠٢/٢٠

٦



MDT FORM 15/2/2026 G

Name	24/2/2026	MRN	1060/2025
Age	34yr	ID	
Menopausal status	premenopausal	Phone	01112340930
NRS SCORE		Marital status	Married
BMI		Family history	-ve
CO morbidity	not ID or HTN	REASON OF MDT	MB C

Clinical Data

BX:				R L  TNM
ER:	PR:	HER 2:	KI67:	
+	+	-ve		
Sonomammography:				
Mets Workup: PET/CT → DP of bone mets				

Recommendations: A Case of ♀ pt @ 34yo @ Rt Breast Ca. At MPM 10/2024 then imaging post operation discovered bone mets palliative. RTH + 6cyd AC then AI + Zoladex + Zometa last PET/CT → DP

MDT OUTCOME & PLAN

for CDK4/6 + fulvestrant + Zoladex + Zometa

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

7011

Patient Name	: الأستاذ / رباح نادي ضاحي	Request No.	: 13004821
Patient No.	: 29207272400329	Request Date	: 02/02/2026
Path No.	: 300/26	Verification Date	: 17/02/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 33 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data

Suspicious left axillary lymph node after right simple mastectomy.

Nature of specimen

Tru cut needle biopsy collected on 02/02/2026, at 11:40 a.m.

cold ischemia time is 10 seconds

Gross examination

Multiple(4) grey white tissue cores ranging in size from 0.2 cm up to 1.5 cm

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:

HER2/NEU (4B5), KI67 (30-9), PR (1E2), ER (SP1), SISH.

Microscopic examination

* Histological type: Nodal tissue cores with metastatic deposits of breast origin, focally necrotized

* IPT (Positive control showed appropriate reactivity):

* GATA3: Diffuse positive nuclear staining.

* ER: Strong nuclear reaction in 90% tumor cells (Allred score 8/8)

* PR: Strong nuclear reaction in 90% tumor cells (Allred score 8/8)

* HER2: Moderate complete membrane reaction in >10% of tumor cells (2+)

* SISH:

-Average HER2 copy No.:1.95

-HER2/CEP17 ratio:1.18

* Ki 67: 40%

Diagnosis

Verified by:

Dr.Heba Hasan

Printing Date : 22/02/2026 10:34:19

Page 1 of 3

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / رباح نادي ضاحي
Patient No. : 29207272400329
Path No. : 300/26
Sex : Female
Age : 33 Y
Referring Doc :

Request No. : 13004821
Request Date : 02/02/2026
Verification Date : 17/02/2026
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Left suspicious axillary lymph node , Tru cut needle biopsy
-Metastatic deposits of breast origin , focally necrotized.
IPT:

- * ER: Positive (score 8/8).
- * PR: Positive (score 8/8).
- * HER2:Equivocal (score +2)
- * SISH: Not Amplified
- * Ki67: 40 %.
- * Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells .IHC 0+:as defined as incomplete membrane reaction in less than 10 % of tumor cells . . IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is

Verified by :
Dr.Heba Hasan



Patient Name	: الأستاذ / رباح نادي ضاحي	Request No.	: 13004821
Patient No.	: 29207272400329	Request Date	: 02/02/2026
Path No.	: 300/26	Verification Date	: 17/02/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 33 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

less than 4. (Recounting give the same result).

- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).

- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result). Presidential Initiative for

Women's Health- Pathology Lab Templates

- Group V (Negative): HER2/CEP17 ratio is less than 2

Signature

Dr.Walaa Ibrahim Ghoneim

Dr.Heba El-Shafeey

Verified by :

Dr.Heba Hasan

Printing Date : 22/02/2026 10:34:19

Page 3 of 3

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name : Rabah Nadi Dahi Hassan

Date : 19-01-2026 09:39

Branch : Nasrcity

Medical No. : 295143

Ref.Physician Prof : Goustav Roussey
Egypt

Ins.Company : التأمين الصحي

Whole Body (FDG) PET/CT Scan

Clinical question:

- 33-year-old lady with history of managed right breast cancer (IDC) underwent mastectomy in 20/10/2024 and on hormonal therapy.
- PET/CT is requested for status evaluation.

Procedure:

- Dose/Radiopharmaceutical: 4 mCi ^{18}F -FDG administered via a peripheral upper limb vein.
- Blood glucose level: 81 mg/dL.
- Imaging was performed on an integrated PET/CT scanner approximately 60 minutes after injection of the radiopharmaceutical.
- CT without contrast images done for attenuation correction and an anatomic localisation were obtained followed by PET images from the vertex of the skull to the calf region.
- PET/CT images were reviewed in the transaxial, coronal and sagittal planes. The Standardised Uptake Value, maximum variant (SUV_{max}) was calculated within regions of interest whenever needed.

Findings:

The prior PET/CT DICOM images are not available for comparison.

Chest (including both breasts and relevant mammary LNs):

- Evidence of right mastectomy with no definite FDG-avid focal lesions or masses at the operative bed.
- Unremarkable appearance of the left breast.
- FDG-avid enlarged left axillary lymph node, measuring ~ 3 cm with increased trace uptake eliciting $\text{SUV}_{\text{max}} \sim 11.9$.
- Few enlarged metabolically active mediastinal right paratracheal, retrocaval, left internal mammary, subcarinal and right hilar LNs, reaching 1.5 cm and eliciting $\text{SUV}_{\text{max}} \sim 3.3$.
- Few tracer starved sub-cm bilateral lung nodules.
- No other enlarged or dense tracer fixing mammary related LNs.
- No lung masses or parenchymal infiltrates.
- No pleural or pericardial effusion.
- Normal tracer distribution over the myocardium.

Call Center **19656**

Name : Rabah Nadi Dahi Hassan

Date : 19-01-2026 09:39

Branch : Nasrcity

Medical No. : 295143

Ref.Physician Prof : Goustav Roussey
Egypt

Ins.Company : التأمين الصحي

Head and neck:

- The brain looks normal with no abnormal focal metabolic activity.
(N.B: it may kindly be noted that some brain lesions not to be apparent on PET/CT and MRI is better for local assessment, if clinically warranted).
- Normal FDG uptake pattern is seen at other soft tissues, glandular structures, oropharynx, larynx and salivary glands.
- No adenopathy or nodal hypermetabolism in the neck.

Abdomen and pelvis:

- Average-sized liver with **multiple FDG-avid hypodense hepatic focal lesions, reaching 1.4 cm and eliciting SUV max ~ 3.7.** No IHBR dilatation could be detected.
- No significant pathologically enlarged or metabolically active pelvi-abdominal lymph nodes.
- Normal spleen, pancreas, kidneys, suprarenals and pelvic structures.
- No ascites.

Musculoskeletal:

- **Multiple metabolically active mixed lytic/sclerotic osseous lesions, the most active (at DV12) eliciting SUV max ~ 9.4.**
- No other definite blastic or lytic bony lesions could be detected all over the surveyed skeleton.

Impression:

Whole body (FDG) PET/CT study is showing:

- Status post right mastectomy with no definite FDG-avid focal lesions or masses at the operative bed **that could cause** active tumor biology.

Name : Rabah Nadi Dahi Hassan

Date : 19-01-2026 09:39

Branch : Nasrcity

Medical No. : 295143

Ref.Physician Prof : Goustav Roussey
Egypt

Ins.Company : التأمين الصحي

- FDG-avid nodal, HFLs and osseous deposits as cribbed, likely metastatic.
- Few bilateral tiny tracer starved lung nodules.
- No other FDG-avid focal lesions could be detected.
- Follow up FDG PET/CT is advisable as clinically warranted.

Best regards,

Dr. Mohammed Akl, M.Sc

MD

Dr. Alaa Nagi,

Nuclear Medicine

Dr. Mohamed Akl

Dr. Alaa Nagy

Call Center **19656**

Name : Rabah Nadi Dahi Hassan
Branch : Maadi
Ref.Physician Prof : Goustav Roussey Egypt

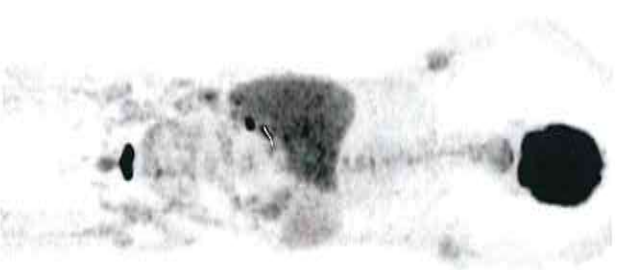
Date : 12-09-2026 07:49
Medical No. : 295143
Ins.Company : التأمين الصحي

Whole Body F18- FDG PET/CT study

Clinical history:- 34 year old female patient , presenting with right breast cancer underwent MRM received CTH , RTH and on hormonal referred for PET CT study
Type of the study :follow up 15/6/2026

rocedure:

- ✗ **Radiopharmaceutical:** 18F FDG, 3.2 mCi I.V.
- ✗ **Blood glucose level at time of 18F-FDG injection:** 87 mg/dL.
- ✗ **Time from 18F-FDG injection to scan:** 60 min
- ✗ **Technique :**MDCT diagnostic examination was taken for attenuation correction anatomic localization followed by PET images from the skull vault to the mid thigh were obtained.
- ✗ Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.
- ✗ **Patient's height:** 150 cm **patient's weight:**65 kg
- ✗ **SUV average of reference hepatic activity=**1.4 .
- ✗ **Measurements are correlated to body weight.**



PET/CT FINDINGS:-

Head and Neck:

- No metabolically active cervical LNs noted.
- The brain exhibited normal FDG bio-distribution
- Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- Normal thyroid uptake

Chest:

- Clear surgical right mastectomy bed with no FDG avid lesions seen
- Morphological regression with complete metabolic resolution as regard the previously noted metabolic active left axillary LN measures 17x15 mm
- Complete resolution of the metabolic active mediastinal LN, internal mammary and right supraclavicular LN
- Still noted bilateral non FDG avid pleural effusion mostly on the right side



Name : Rabah Nadi Dahi Hassan

Date : 12-09-2026 07:49

Branch : Maadi

Medical No. : 295143

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : التأمين الصحي

- Normal FDG uptake is seen throughout the both lung lobes with no FDG avid lung nodules or masses detected.
- There are no pleural or pericardial abnormalities.
- Physiologic FDG uptake is noted in the myocardium.

Abdomen and Pelvis:

- Stationary morphological course as regard the mottled liver appearance of multiple innumerable hypodense hepatic focal lesions the most avid seen at segment IV and V with SUV max of 7.7 (compared to SUV max of 10)
- No evidence of splenic FDG avid focal lesions.
- No metabolically active pelvi-abdominal LNs noted.
- There are no FDG avid adrenal nodules.
- No evidence of FDG avid peritoneal nodules
- Physiologic FDG excretion is seen in the kidneys and bladder.



Musculoskeletal:

- Still noted multiple mixed lytic and sclerotic bone lesions seen involving the axial and the appendicular skeleton reaching currently SUV max of 2.5 (compared to 5)

Conclusion:

- Follow up PET/CT study revealed still clear right mastectomy bed with no FDG avid lesions to account for residual or recurrence
- Metabolic and morphological regression as regard the left axillary LN
- Complete resolution of the mediastinal , right supra clavicular , internal mammary LNs
- Stationary morphological course as regard the multiple osseous lesions with marked metabolic regression
- Stationary morphological course as regard the multiple hypodense hepatic focal lesions with regressive metabolic course

Thanks for referral;

Dr. NORA NABIL, MD

Lecturer of Radio-diagnosis,

ASUH

Name : Rabah Nadi Dahi Hassan
Branch : Maadi
Ref.Physician Prof : Goustav Roussey Egypt

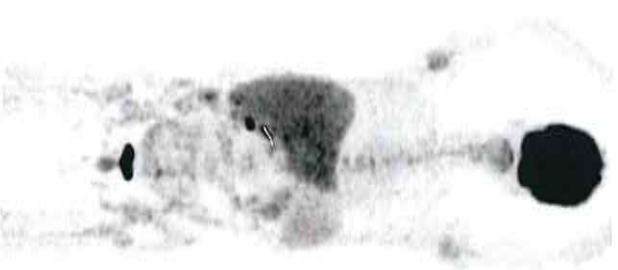
Date : 12-09-2026 07:49
Medical No. : 295143
Ins.Company : التأمين الصحي

Whole Body F18- FDG PET/CT study

Clinical history:- 34 year old female patient , presenting with right breast cancer underwent MRM received CTH , RTH and on hormonal referred for PET CT study
Type of the study :follow up 15/6/2026

rocedure:

- ✍ **Radiopharmaceutical:** 18F FDG, 3.2 mCi I.V.
- ✍ **Blood glucose level at time of 18F-FDG injection:** 87 mg/dL.
- ✍ **Time from 18F-FDG injection to scan:** 60 min
- ✍ **Technique :**MDCT diagnostic examination was taken for attenuation correction anatomic localization followed by PET images from the skull vault to the mid thigh were obtained.
- ✍ Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.
- ✍ **Patient's height:** 150 cm **patient's weight:**65 kg
- ✍ **SUV average of reference hepatic activity=**1.4 .
- ✍ **Measurements are correlated to body weight.**



PET/CT FINDINGS:-

Head and Neck:

- No metabolically active cervical LNs noted.
- The brain exhibited normal FDG bio-distribution
- Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- Normal thyroid uptake

Chest:

- Clear surgical right mastectomy bed with no FDG avid lesions seen
- Morphological regression with complete metabolic resolution as regard the previously noted metabolic active left axillary LN measures 17x15 mm
- Complete resolution of the metabolic active mediastinal LN, internal mammary and right supraclavicular LN
- Still noted bilateral non FDG avid pleural effusion mostly on the right side



Name : Rabah Nadi Dahi Hassan

Date : 12-09-2026 07:49

Branch : Maadi

Medical No. : 295143

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : التأمين الصحي

- Normal FDG uptake is seen throughout the both lung lobes with no FDG avid lung nodules or masses detected.
- There are no pleural or pericardial abnormalities.
- Physiologic FDG uptake is noted in the myocardium.

Abdomen and Pelvis:

- Stationary morphological course as regard the mottled liver appearance of multiple innumerable hypodense hepatic focal lesions the most avid seen at segment IV and V with SUV max of 7.7 (compared to SUV max of 10)
- No evidence of splenic FDG avid focal lesions.
- No metabolically active pelvi-abdominal LNs noted.
- There are no FDG avid adrenal nodules.
- No evidence of FDG avid peritoneal nodules
- Physiologic FDG excretion is seen in the kidneys and bladder.



Musculoskeletal:

- Still noted multiple mixed lytic and sclerotic bone lesions seen involving the axial and the appendicular skeleton reaching currently SUV max of 2.5 (compared to 5)

Conclusion:

- Follow up PET/CT study revealed still clear right mastectomy bed with no FDG avid lesions to account for residual or recurrence
- Metabolic and morphological regression as regard the left axillary LN
- Complete resolution of the mediastinal, right supra clavicular, internal mammary LNs
- Stationary morphological course as regard the multiple osseous lesions with marked metabolic regression
- Stationary morphological course as regard the multiple hypodense hepatic focal lesions with regressive metabolic course

Thanks for referral;

Dr. NORA NABIL, MD

Lecturer of Radio-diagnosis,

ASUH